This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Finasteride for the treatment of Androgenetic Alopecia

Patient Group Direction, version 005, issued 30 September 2026. Supersedes Version 004, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE CKS Guidelines for Androgenetic Alopecia

Definition

- **Androgenetic alopecia (AGA)** is a common, progressive, non-scarring form of hair loss:
    - **Men**: Recession of the frontal hairline and thinning at the vertex.
    - **Women**: Diffuse thinning over the crown with preservation of the frontal hairline.

Diagnosis

- Diagnosis is **clinical**, based on:
    - Pattern and distribution of hair loss.
    - Gradual onset, usually starting after puberty.

- No evidence of scalp inflammation or scarring.
- Investigations (e.g. hormone levels or biopsy) are **not routinely required**, unless:
  - Atypical pattern or rapid progression.
  - Signs of hyperandrogenism in women (e.g. hirsutism, acne, menstrual irregularities).

Management

1. Reassurance and Support

- AGA is benign but can cause significant psychological distress.
- Provide **realistic expectations**: treatments may slow or partially reverse hair loss but are **not curative**.

2. Treatment Options

Men:

- **Topical minoxidil 5%**:
  - Apply twice daily to affected areas.
  - Can **slow hair loss and stimulate regrowth**.
  - Effects are gradual (visible improvement in 2–4 months).
- **Oral finasteride 1 mg daily**:
  - A 5-alpha-reductase inhibitor that reduces scalp DHT.
  - May improve hair density and slow progression.
  - Contraindicated in women and children.
  - Side effects: reduced libido, erectile dysfunction, breast tenderness.

Women:

- **Topical minoxidil 2% or 5%**:
  - First-line treatment, used once or twice daily.
  - Often continued long-term for sustained benefit.
- **Oral treatments** (off-label and specialist use only):
  - Anti-androgens (e.g. spironolactone or cyproterone acetate) may be considered in women with hyperandrogenism.
  - These require **specialist input** due to risks and monitoring requirements.

3. Other Considerations

- Cosmetic options: camouflage products, hairpieces, and wigs.
- Surgical options: hair transplantation (private services).
- No evidence to support efficacy of nutritional supplements unless deficiency is identified.

When to Refer

- Uncertain diagnosis or atypical presentation.

- Sudden or scarring hair loss.
- Suspected underlying endocrine disorder in women.

Patient Group Direction

for the supply/administration of

Finasteride 1mg

for the treatment of

Androgenetic Alopecia

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Finasteride 1 mg is indicated for the treatment of androgenetic alopecia (male pattern hair loss) in men aged 18 to 65 years to increase hair growth and prevent further hair loss.
Inclusion criteria	- Male patients aged 18 to 65 years. - Clinical diagnosis of androgenetic alopecia. - No contraindications to treatment. - Patient has provided informed consent and understands risks/benefits.
Exclusion criteria	- Female patients (Finasteride 1 mg is not indicated). - Known hypersensitivity to finasteride or any component of the formulation. - History of liver disease. - Current use of 5-alpha-reductase inhibitors for other conditions. - Suspected prostate cancer or raised PSA under investigation.
Cautions	- Finasteride can affect PSA levels; inform patient to tell any clinician conducting PSA tests. - Tablets should not be handled by women who are or may become pregnant (risk of fetal harm). - Treatment should be reassessed if no improvement after 12 months. - Advise on psychological side effects and to report any mood changes. - The use of a condom is recommended if a female partner is pregnant or likely to become pregnant - Finasteride is excreted in semen and it is not known if a male fetus may be adversely affected if its mother is exposed to the semen of a man being treated with finasteride. - Breast cancer has been reported in men taking finasteride 1mg during the post-marketing period. Patients should be advised to promptly report any changes in their breast tissue such as lumps, pain, gynaecomastia or nipple discharge. - Mood alterations including depressed mood, depression and, less frequently, suicidal ideation have been reported in patients treated with finasteride 1 mg. Patients should be monitored for psychiatric symptoms and if these occur, treatment with finasteride should be discontinued and the patient advised to seek medical advice. - Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine. - Although, animal studies did not show relevant negative effects on fertility, spontaneous reports of infertility and /or poor seminal quality were received post-marketing. In some of these reports, patients had other risk factors that might have contributed to infertility. Normalisation or improvement of seminal quality has been reported after discontinuation of finasteride.
Actions if patient is excluded or declines	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP

treatment	as appropriate

Details of the medicine

Name, form and strength of medicine	Finasteride 1mg tablets
Legal category	POM
Storage	Do not store above 25°C. Store in original package to protect from moisture and light.
Route/method of administration	Oral administration.
Dose and frequency	1 mg orally once daily, with or without food
Quantity to be administered and/or supplied	Up to 12 months of treatment per supply, and no more than 12 months without a review. The first review should be at 3 to 6 months, so the first supply should not normally exceed 6 months. Reassess if there is no improvement after 12 months.
Maximum or minimum treatment period	Minimum 3 to 6 months of continuous treatment to assess effectiveness.
Adverse effects	Reduced libido, erectile dysfunction, ejaculation disorders, breast tenderness or enlargement, depression. Rare reports of male breast cancer. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as necessary.

Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication and the patient card included within the pack.
Follow-up advice to be given to patient or carer	- To seek medical advice if adverse effects experienced or becomes systemically very unwell. - Do not take more than the recommended dose it will not improve results - Damage to the scalp could occur as a result of loss of protection against ultraviolet light, cold and mechanical injury - Drug treatments can slow down hair loss and, to a lesser extent, produce regrowth of lost hair; however, complete reversal of hair loss is never achieved - Clinical studies demonstrate beneficial effects of finasteride within males aged 18-41 years. Benefits may be less in patients above these ages. - Hair loss usually involves the front and sides of the scalp initially, then progresses towards the back of the head. - Continuous use for 3–6 months is required before evidence of stabilization of hair loss can be expected. - Peak hair growth is achieved after 2 years, but treatment must be continued to maintain results. - If treatment is stopped, the beneficial effects begin to reverse by 6 months and return to baseline by 9–12 months - Be aware that hair loss can lead to adverse psychosocial effects such as impaired self-esteem, embarrassment and frustration

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE

- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th Jan 2026

Version and change record

Version: v005

Issued: 30 September 2026

Supersedes: Version 004, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v004

Issued: 14 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from administration to supply; the tablets are supplied.

Cautions: finateride typo corrected.

Quantity row: 12 months is stated as the maximum supply without a review, with the first review at 3 to 6 months and the 12-month reassessment carried in from the cautions.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 25 January 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

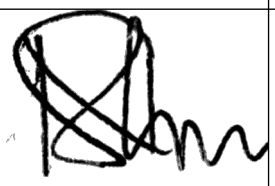
Changes in this version:

First reissue of this document. One document serves both the 'alopecia-minoxidil' and 'hair-loss' catalogue entries.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v005, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.